



Engagement & Audit Guide

Haven Paediatric Centre — for Tito Ipinmoye, co-lead

Prepared by Dr Debo Odulana, Consult for Africa · Internal — not for the client

Welcome to Haven. You're championing this one alongside me, and this guide is everything you need to run it well: the context, the sensitivities that matter most, and a week-by-week playbook for the diagnostic audit. Read section 2 before anything else — on this engagement, *how* we carry ourselves matters as much as *what* we find.

1. Your role, and how we split it

You are the engagement lead on the ground. You own the day-to-day: the schedule, the point-of-contact relationship at Haven, issuing and chasing the data request, running the survey, driving interviews and observation, and pulling the findings together. I hold partner oversight, the board relationship, the commercial conversation, and the senior clinical and strategic judgement. When in doubt, over-communicate to me — daily quick check-ins during fieldwork.

Route to Debo, always: anything about money or payment; anything involving the board or the owners' relationships; anything touching the May 2026 incident; and any decision that changes scope or price.

2. Read this first — the sensitivities that govern everything

This is not a normal turnaround. Internalise these before you speak to anyone at Haven.

The incident — how we hold it

In May 2026 Haven lost a child. The child was referred in already critically ill and may not have been saveable under any circumstances; the loss is not attributable to the facility. It is what prompted leadership to call us.

We do not raise it. Ever — in writing or in conversation. Not in the data request, the interviews, the report, or a message. If a staff member raises it, listen with care, do not probe for blame, and move on.

- **Every client-facing word is forward-looking:** we are here because a strong young facility is putting world-class systems under its growth — not because something went wrong. That is the honest, respectful and legally sound framing.
- **Why it's this strict:** documents travel and are discoverable. A written record linking a death to any facility gap is a liability for Haven and for us, and it reopens a wound for a grieving board — one owner is a former co-founder of mine. The audit stands entirely on its own without ever invoking the loss.

The relationship & governance

- I was **invited to Haven's board from inception but do not formally sit on it**. Say “invited to the board,” never “sits on the board.” The board is not yet formally constituted.
- CFA is a **paid partner** to a company I'm close to — handled with total transparency: pricing disclosed to all owners, clean fixed fees, **no success fee**. Never imply special treatment or leverage.

The commercials & the thesis

- Fee is **N9.3M** (negotiated from a standard N17M — a large concession already made). Do not use the discount as leverage or discuss it as a favour. If payment comes up, it comes to me.
- This is a **culture and systems build, not a set of process patches**. Strong routines, standards of work and incentives are what make safe, efficient care self-sustaining. “**Culture is the multiplier**” is the line — everything we recommend ladders up to it.

3. What Haven is



A ~15-month-old private facility in GRA Ikeja: 5 general-paediatric beds + a 3-bed NICU. Owners: Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, and Dr Odedina (consultant neonatologist). It funds its own salaries and has a real local reputation — a genuine achievement this young. It is now scaling into more complex care (NICU), exactly when the systems beneath it must be deliberate rather than assumed.

The three business truths shaping our work:

- 1. The cash problem is a working-capital problem, not a profit problem.** ~N4.2M is locked in HMO receivables (Leadway + NEM ≈ a full period's revenue) and ~N2.77M in pharmacy stock — roughly N7M tied up. Freeing it is our fastest, most visible win.
- 2. NICU is the growth engine** (≈N3M deposit per admission, 3 beds) — and scales safely only once governance and standards are in place. Fix the foundation, then fill NICU.
- 3. The reporting layer is immature, and that itself is a finding** — visit counts don't reconcile, the receivables table doesn't foot, a “201% pharmacy profit” is a markup not a margin. A senior operations leader (recruited via CadreHealth) is part of the answer.

4. The audit playbook — four weeks

Your toolkit is already built, in *docs/*: the Information & Data Request (the branded PDF goes to their point person), the staff safety-culture survey (online form), the fieldwork kit (your interview guides + observation checklist), and the crash-cart standard (the day-one quick win).

Week	Where	What you drive
1	Kick-off + remote	Get a named point person (push for Head of Operations). Issue the data request; flag the 12 Week-1 critical items. Launch the survey. Put the crash-cart standard live. Start the Leadway/NEM receivables push. Begin reconciling the management accounts.
2	On site	Interviews (owners, clinical lead, neonatology, matron, pharmacy, front desk, accounts) using the fieldwork guides. Observation across shifts. SOP walk-throughs. NICU readiness.
3	On site + remote	Finance & working capital; procurement & inventory (physical spot-counts vs system); HMO economics; reporting-integrity reconciliation. Close the survey.
4	Remote + board	Synthesise into a prioritised, costed plan. Present the two deliverables in a board working session.

Two deliverables: (1) Operational & Clinical Governance Audit Report; (2) Working Capital & Receivables Recovery Plan — walked through with the board, never just emailed to file.

5. How we work — the posture

- **Collaborative, not audit-by-ambush.** The team is a partner in the findings, not a target. Say so at the start of every interview. We diagnose a system, never blame a person.
- **“Nothing here is a test.”** Partial or messy records are themselves a finding. Take what exists.
- **Observe the real operating day** — a handover, a drug round, a patient from arrival to billing.
- **Reconcile numbers against each other.** Where visit counts, receivables and stock disagree is where the story is.
- **Protect the survey's anonymity fiercely.** Haven is small; report only grouped totals, never anything that could identify a respondent, and never let management see raw responses.
- **Confidentiality throughout** — one secure, read-only shared folder; data used only for this audit.

6. What to look for

- **Free the cash:** size the receivables precisely, get Leadway/NEM recovery moving in week 1, quantify how much of the N2.77M stock is releasable working capital.



- **Real pharmacy margin** vs the markup illusion; where margin leaks; the case for vendor-managed inventory (we lean toward Medbury Pharma).
- **Reporting you can trust:** what it takes to give the board numbers that reconcile.
- **Culture & incentives as the lever:** the commission structure and JDS/KPIs awaiting board approval are the exact tools a strong culture runs on; the survey shows where the gaps are.
- **NICU safe-scaling** and **shift-level routines** (crash-cart check, handover) — the visible proof the culture is changing.

7. Practical logistics

- **Cadence:** daily 10-minute check-in with me during fieldwork; a short written end-of-week summary.
- **Escalate to me:** payment, board/owner relationships, the incident, scope/price, anything that feels off.
- **Comms tone with Haven:** warm, precise, confident, forward-looking — the calm, credible partner, never the auditor come to catch them out.

8. Do / Don't

Do	Don't
Frame everything forward: building systems for growth	Reference the May 2026 loss, in writing or speech
Say "Dr Odulana was invited to the board"	Say or imply he "sits on the board"
Treat staff as partners in the findings	Make anyone feel blamed or tested
Report survey results as grouped totals only	Let management see individual responses
Route money / board / incident matters to Debo	Negotiate price or discuss the discount yourself
Lead with "culture is the multiplier"	Sell the work as a set of process fixes

Welcome aboard, Tito. Do this one well and it becomes the template for how CFA runs a facility turnaround. Any question at all, ask me early rather than late. — Debo